Statins and psoriasis: Position statement by the Psoriasis Task Force of the European Academy of Dermatology and Venerology

Abstract

Background: Psoriasis is associated with increased mortality, primarily due to cardiovascular disease (CVD), with risk escalating in a dose-response manner according to psoriasis severity. Statins reduce all-cause mortality without significant adverse effects in the general population. Underuse of statins in patients with psoriasis may stem from inadequate cardiovascular risk assessment.

Objectives: To assist dermatologists in systematically managing dyslipidemia in psoriasis—often outside routine dermatological practice—and to support decision-making regarding patient referral and treatment.

Methods: The Psoriasis Task Force of the European Academy of Dermatology and Venereology conducted a two-phase study. Phase one involved a systematic review to identify key concepts and recommendations based on existing guidelines. Phase two used a two-round Delphi process to evaluate recommendations not directly derived from guidelines.

Results: A final list of 47 consensus-based concepts and recommendations was developed for dermatologists managing moderate-to-severe psoriasis. These were grouped into six domains:
- 6 statements on cardiovascular risk and psoriasis
- 6 on low-density lipoprotein cholesterol (LDL-c) and statin benefits in psoriasis
- 8 on cardiovascular risk assessment

- 3 on non-invasive cardiovascular imaging
- 3 on LDL-c thresholds
- 8 on statin prescription
- 10 on statin treatment follow-up
- 3 on referral to other specialists

Conclusions: Implementation of this position statement will enable dermatologists to better manage dyslipidemia and contribute to reducing cardiovascular risk in patients with psoriasis.

Introduction

Psoriasis is linked to accelerated atherosclerosis and an elevated risk of cardiovascular (CV) events. This association is not fully explained by traditional CV risk factors such as smoking, hypertension, and hyperlipidemia; immune-mediated inflammation also plays a key role in increasing CV risk. Commonly used CV risk prediction tools have limitations, as they were not designed for inflammatory conditions like psoriasis and often underestimate actual risk.

Recognizing the connection between psoriasis and CVD offers an opportunity for education and preventive care, potentially improving both overall health and quality of life. Despite this, interventions such as statin therapy remain underutilized. For example, only 24% of psoriasis patients eligible for statins for primary prevention are actually receiving them.

This is concerning because statin therapy safely reduces major vascular events, even in individuals with a 5-year CV risk below 10% (and separately below 5%). Each 1.0 mmol/L reduction in LDL cholesterol translates to 11 fewer major vascular events per 1,000 patients treated over 5 years—a benefit that far outweighs potential risks. Statins are equally effective in reducing LDL and preventing CV events in patients with and without psoriasis. Additionally, meta-analyses suggest that oral statins may improve psoriasis severity, particularly in those with more severe disease.

Current guidelines now consider chronic inflammatory conditions like psoriasis as CV risk enhancers when estimating 10-year CV risk. However, psoriasis alone does not currently constitute a standalone indication for lipid-lowering therapy. In patients with established CVD, statin use is clearly indicated. In contrast, the role of statins for primary prevention in psoriasis remains less defined and requires further clarification.

Various strategies have been proposed to address CV risk in psoriasis, but standardized approaches integrating dermatological and cardiovascular care are still evolving.